

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

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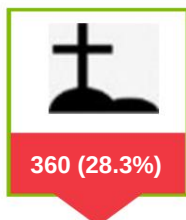
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°044/MVB_27/06/2026

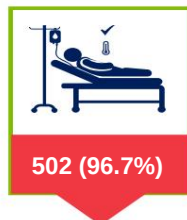
Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (35)	- Ituri (23/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki and Mandima. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa
Reporting date	June 27, 2026
Publication date	June 28, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



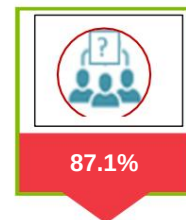
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 3 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- Forty-seven (47) new confirmed cases, including 12 deaths, were reported during the day on June 27, 2026, in the provinces of Ituri (45 cases, including 11 deaths) and North Kivu (2 cases, including 1 death). These confirmed cases come from the health zones of Rwampara (19), Bunia (6), Mongbwalu (5), Nizi (3), Komanda (3), Drodoro (2), Bambu (2), Damas (1), Lita (1), Mangala (1), Mandima (1), Nyankunde (1) in Ituri and Katwa (2) in North Kivu.
- On the other hand, 10 deaths were recorded among patients with Ebola virus disease in the Ebola Treatment Centers (ETCs) of Ituri province (5 in Mongbwalu, 2 in ETC Elihya/ZS Bunia, 1 in ETC Nyankunde, 1 in ETC Komanda and 1 in ETC ISTM/ZS Rwampara).
- A new health zone was affected as of June 27, 2026, namely **Mandima** (Ituri province). Thus, **23 (63.9%) out of the 36** health zones in this province are currently affected.
- Fifteen (15) patients** with Ebola virus disease in Ituri province have been declared cured after obtaining negative control tests.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

The Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million inhabitants, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km

from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola outbreak, which was then raging in North Kivu. The current outbreak, caused by Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million inhabitants, with massive internal displacements and cross-border flows to neighboring countries.

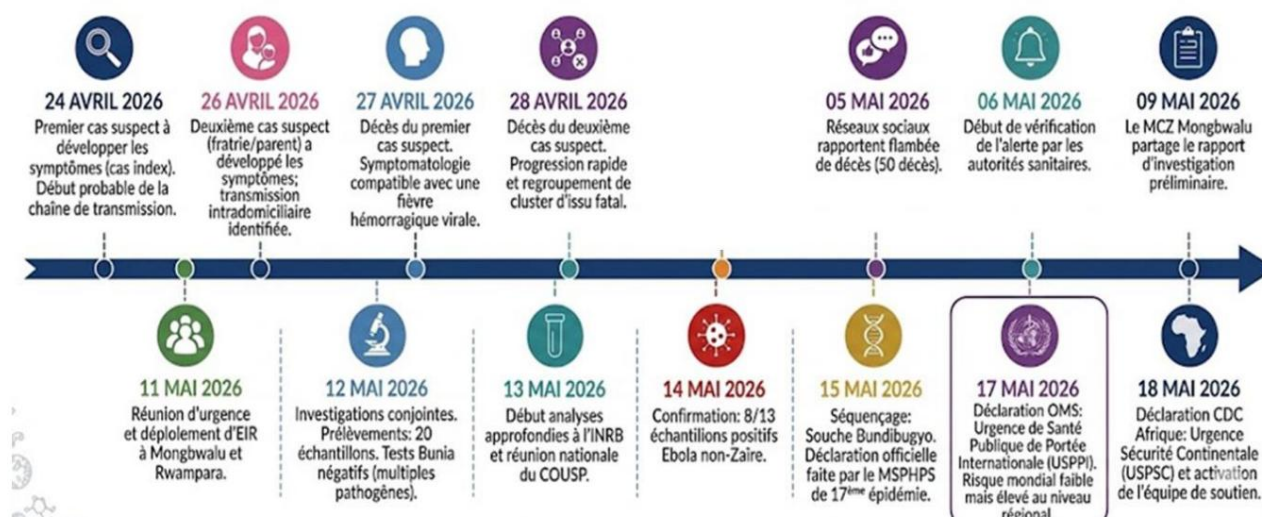


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 27, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of June 27, 2026.

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New case confirmed (24h)
Ituri	1165	301*	25.8%	23 out of 36 (63.9%)	45
North Kivu	106	58	54.7%	11 out of 34 (32.3%)	2
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	1274	360	28.3%	35 out of 104 (33.6%)	47

* 10 deaths were recorded among patients with Ebola virus disease (previous confirmed cases) in the Ebola treatment centers/treatment centers in Ituri.

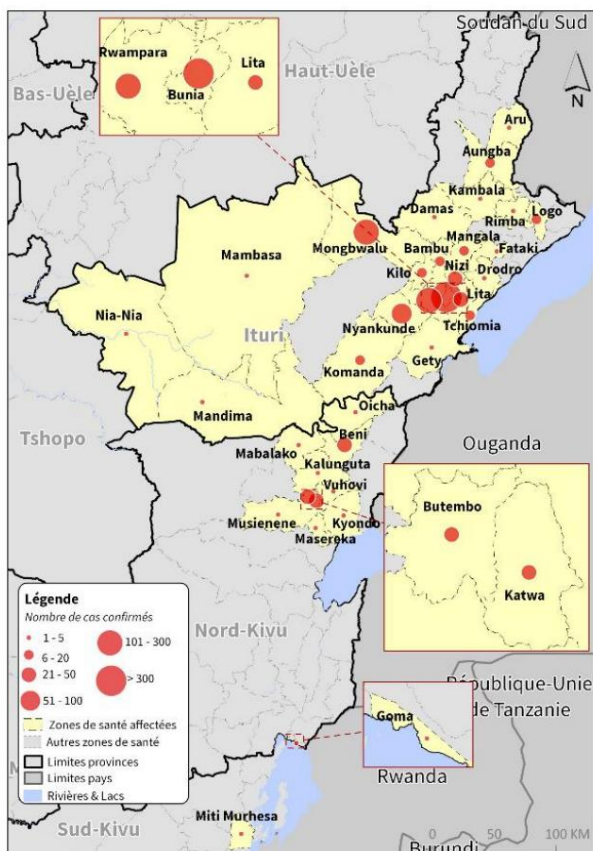
At the end of June 27, 2026, a new health zone (Mandima Health Zone) was affected in Ituri province. The number of affected health zones has risen from 34 to 35 out of 104 across the three affected provinces.

Since May 26, 2026, the province of South Kivu has not recorded any confirmed cases, suggesting an absence of recent transmission.

3.2 Distribution by health zone (Cumulative as of 27/06/2026)

- **Ituri Province (1,165 cases):** Bunia (338), Rwampara (282), Mongbwalu (258), Nyankunde (94), Nizi (37), Lita (27), Mangala (20), Bambu (16), Komanda (16), Tchomia (11), Kilo (9), Logo (7), Aungba (6), Nia-Nia (7), Damascus (5), Rimba (3), Aru (3), Drodoro (3), Mambasa (2), Gety (1), Kambala (1), Fataki (1) and Mandima (1). Seventeen (17) additional cases are awaiting precise breakdown by health zone in the province.
- **North Kivu Province (106 cases):** Katwa (37), Butembo (33), Beni (22), Oicha (3), Kyondo (3), Kalunguta (2), Musienene (2), Goma (1), Masereka (1), Vuhovi (1) and Mabalako (1).
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Epidemic mapping



Figures 2 and 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 27, 2026.

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (June 27, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative nonconfirmed deaths (n)	Case fatality rate (CFR, %)
ITURI	1165	301	25.8%
Bunia	338	71	21.0%
Rwampara	282	52	18.4%
Mongbwalu	258	118	45.7%
Nyankunde	94	13	13.8%
Nizi	37	9	24.3%
Lita	27	7	25.9%
Komanda	16	5	31.3%
Bamboo	16	4	25.0%
Kilo	9	1	11.1%
Mangala	20	9	45.0%
Tchomia	11	0	0.0%
Damascus	5	0	0.0%
Aungba	6	2	33.3%
Rimba	3	0	0.0%
Aru	3		33.3%
Mambasa	2		50.0%
Logo	7	1	0.0%
Drodro	3	1	100.0%
Gety		0	0.0%
Kambala			100.0%
Nia-Nia*	1	3	57.1%
Fataki		0	0.0%
Mandima	1 7 1 1	1	0.0%
Other areas not yet identified	17	4 0 0 0	0.0%
NORTH KIVU	106	58	54.7%
Butembo	33	14	42.4%
Katwa	37	23	62.2%
Blessed	22	13	59.1%
Oicha		2	66.7%
Kalunguta	3 2	1	50.0%
Kyondo	3	2	66.7%
Goma	1	0	0.0%
Masereka	1	0	0.0%
Vuhovi	1		100.0%
Mabalako	1	1	0.0%
Musienene	2	0	100.0%
SOUTH KIVU	3	2 1	33.3%
Miti-Murhesa	3	1	33.3%
TOTAL 28.3%	1274	360	

*As of June 25, 2026, 2 contacts listed around a confirmed case in the Nia-Nia health zone were found to be symptomatic in the Wamba health zone (Haut-Uélé province), they were tested and came back positive for Ebola virus disease.

In the past 24 hours, **47 new confirmed cases, including 12 deaths**, have been recorded in the provinces

The cases in Ituri (**45 cases, including 11 deaths**) and North Kivu (**2 cases, including 1 death**) highlight the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) outbreak in the Democratic Republic of Congo. The cumulative total now stands at **1,274 confirmed cases** and **360 deaths**, representing an overall case fatality rate of **28.3%**.

Provincial dynamics and case concentration

Ituri province remains the absolute epicenter of the epidemic, accounting for **91.4%** of cases (n=1,165) and **83.6%** of deaths (n=301), representing a case fatality rate of **25.8%**. The health zones of Bunia (338 cases), Rwampara (282 cases), and Mongbwalu (258 cases) remain the most affected, representing approximately 75.3% of reported cases at the provincial level. Moderate case fatality rates were observed in the health zones of Rwampara (18.4%) and Bunia (21.0%), while the **Mongbwalu** zone exhibited marked severity, with a case fatality rate of **45.7%** (n=301).

(deaths out of 258 cases), suggesting persistent challenges in early management or access to care. It should be noted that 17 additional cases (with no associated deaths) have been reported, while the health zones of origin remain to be identified, which could indicate a geographical expansion requiring further investigation.

a reinforced investigation.

Severity of the epidemic in North Kivu

Since the start of the epidemic, **106 confirmed cases and 58 deaths** have been reported in North Kivu province, representing a **case fatality rate of 54.7 %**. This province presents the most concerning profile in terms of severity, with a case fatality rate higher than the national average. This high case fatality rate is driven by the health zones of **Katwa** (62.2%), **Beni** (59.1%), and **Oicha** (66.7%). The Katwa health zone has the highest number of reported cases (n=37), followed by the Butembo health zone (n=33) with a lower case fatality rate (42.4%). A case fatality rate of 100% was observed in the exceptionally small Vuhovi and Musienene health zones, highlighting the danger posed by the virus in this region and the need for a rapid response.

immediate clinical examination.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Continued holding of coordination meetings at different levels.
- Supervision of the activities of the pillars of the response.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of June 27, 2026

Indicator		Ituri North Kivu		South Kivu	Total
Report (D-1)		7	0	5	12
New Alerts		290	962	19	1271
Total alerts for the day		297	962	24	1283
Alerts investigated		290	962	19	1271
Investigation rate		97.6%	100.0%	79.2%	99.1%
Alerts confirmed	Alive	121	44	4	169
	Deceased	44	26	0	70
Total suspected cases for today		165	70	4	239

For the day of June 27, 2026, **1,283 alerts** were reported across the three provinces, **1,271 (99.1%)**

239 suspected cases were investigated, **including 70 deaths**, which were identified (Table 3).

Table 4. Contact tracing of confirmed cases as of June 27, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	7,235*	6,199	85.7%
North Kivu	1,892*	1,747	92.3%
South Kivu	18	18	100.0%
Total	9,145	7,964	87.1%

* Data currently being validated: harmonization is not yet finalized. Three (3) health zones are experiencing security challenges that are impacting activity monitoring and data completeness. In North Kivu, the monitoring rate was calculated only for health zones that reported (completeness: 80%).

- Continued in-depth investigations and exhaustive listing and systematic follow-up of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and followed per case).
- **260 new contacts** were listed, including 210 in 6 health zones in Ituri province and 50 in 2 health zones in North Kivu province.
- **50 contacts successfully completed their 21-day follow-up** in North Kivu province.

Update on checkpoint and point of entry (PoC/PoE) activities

- Activation of 2 PoC Base Yedi/As Sadiki/ZS Komanda and Banguba/As/ZS Nia-Nia.
- **Ituri** : Seven (7) alerts were notified, including 3 deaths (bodies intercepted). Of the 4 live alerts, 3 were validated and referred to the ESS (Emergency Situations Service).
- **In North Kivu** : 3 live alerts were notified to the Kasindi PoC, two were invalidated after the Secondary screening and 1 validated and referred to CS Bunji.

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of June 27, 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	94.5% (52/55)	100.0% (14/14)	95.7% (66/69)
Number of people who went through PoE/PoC	96,447	55,924	152,371
testing, % of travelers screened,	94.9%	98.0%	96.1%
% of travelers who washed their hands, %	94.0%	98.0%	95.5%
of travelers made aware of the issue	98.0%	98.0%	98.0%
Number of alerts notified	7	3	10
Number of validated live alerts	3	1	4
Number of lifeless bodies intercepted today	3	0	3
Number of problematic contacts intercepted today	0	0	0
Percentage of validated live alerts referred to the CT/CTE	100.0%	100.0%	100.0%
% of the lifeless bodies swabed	100.0%	0.0%	100.0%
Number of positive results of referred suspected cases	0	0	0

4.3. Laboratory

- **Ituri** : 162 samples collected and analyzed (positivity 27.7%; n=45).
- **North Kivu** : 115 samples received, of which 99 were analyzed, 2 tested positive for Ebola virus disease. In addition, 16 samples are currently being analyzed (11 in Beni and 5 in Butembo).
- **South Kivu** : 4 samples were collected and analyzed, all came back negative.

4.4. Infection Prevention and Control (IPC)

Ituri: 18 households identified around cases in the health zones of Bunia, Mongbwalu, Nyankunde, and Komanda, of which 13 were decontaminated in the health zones of Bunia, Mongbwalu, Nyankunde, and Komanda; 4 health establishments (HEs) identified and decontaminated; 29/29 health assessments (HAS) carried out with 14 postponements made up; 18 community swabs carried out in the health zones of Bunia, Mongbwalu, Nyankunde, and Komanda. IPC/WASH assessment of 4 HEs at the Hoho Health Center: 17.7%, PS only: 35.4%, PS Benedicte: 24.2%, CM Lobiko: 62.9%, support provided to 13 HEs, decontamination of 3 HEs, decontamination of 29 means of transport, and briefing of 258 people, as well as 73 community health workers and community leaders; Briefing during the formative supervision of 37 service providers on General information on Ebola virus disease and standard precautions.

North Kivu : Launch of training for 36 service providers including 12 from the EDS team in the Kyondo health zone, 12 in the Butembo health zone and 12 from the Musienene health zone in the Butembo health zone; briefing of ECPs and supervisors of the PCI pillar on data collection tools.

4.5 Holistic care

- **Fifteen (15) patients** were declared cured in Ituri, after obtaining negative control results.
Thus, the cumulative number of recovered patients is **178, including 168 in Ituri, 8 in North Kivu and 2 in South Kivu.**
- **Ten (10) deaths were recorded** among patients with Ebola virus disease (EVD) under care in Ebola treatment centers in Ituri province (5 in Mongbwalu, 2 in Elikya/ZS Bunia, 1 in Nyankunde, 1 in Komanda and 1 in ISTM/ZS Rwampara).
- By the end of the day on June 27, 2026, 774 hot meals had been served, including 146 to the cases
151 confirmed cases, 422 suspected cases, and 55 accompanying persons. Five (5) dry rations were also distributed.

Table 6. Patient movement within healthcare facilities (CTE/CT/CI) as of June 27, 2026

Indicator	Ituri	North Kivu*	South Kivu	Together
Patients in bed (Day -1)	333	134	17	484
Total admissions (24h)	50	20	4	74
Exits (24h)				
Deceased (Suspected/Confirmed) 15 0 Non-cases 14 0 Recovered 15 0 Escaped				16
(Suspected/Confirmed) 1 0 Transferred to General Referral Hospital 0 0 Total				24
discharges 45 0 Patients in isolation (End of Day) 338 21 of which confirmed (NC+AC)				15
192 0 of which suspected 146 21 Overall occupancy rate 90.9% 47.7% *Saturation of				1
isolation units for suspected cases was observed in the health zones of Butembo, Kalunguta,				0
		10		56
		0 0		502
		0		208
		11		294
		143 16 127 138.8%		96.7%

Masereka and Musienene.

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of June 27, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	3 (37.5%)	0	0
Confirmed cases currently being monitored that have received interventions SMSPS	48 (44.8%)	8 (100.0%)	0
New suspected cases that benefited from SMSPS interventions	21 (61.7%)	6 (100.0%)	0
Suspected cases under follow-up that have received interventions SMSPS	51 (45.9%)	27 (100.0%)	1 (100.0%)
Laboratory results have been announced.	6	28 (100.0%)	0
Including positive ones	3	0	0
Number of children separated in daycare who benefited from interventions SMSPS	1	0	0
Number of children separated in the community who benefited from SMSPS interventions	0	0	3
Number of accompanying persons at the CTE who benefited from interventions SMSPS	57	7	0
Number of PPLs who benefited from SMSPS interventions	74	4	7
Number of household and community members who benefited from SMSPS interventions	7	1	0
Number of EDS supported	7	10	0

4.6 Continuity of care

- Supply of medicines to four health zones (Bunia, Nyankunde, Rwampara and Mongbwalu). The zones already supplied continue the distribution of medicines to the targeted health centers, while the Nyankunde health zone is in the process of unpacking the medicines.
- Management of non-cases (children, mothers and other vulnerable individuals) within the framework of continuity of services
- Provision of essential medicines for PMA and PCA in other ZS.

4.7 Risk communication and community engagement

Ituri: Monitoring of community engagement activities led by women leaders in the health zones of Bunia and Rwampara; 33 social mobilization activities (community dialogues and mass awareness campaigns) reaching 4,518 people (1,676 men and 2,842 women) on Ebola prevention measures; community-based surveillance and care, decontamination, and EDS in the health zones of Mongbwalu and Nyankunde; 4 educational talks were organized and 3,376 people were reached; an integrated preparatory meeting

MVE was organised for the benefit of 45 motorcycle taxi parking managers in Bunia and Rwampara; 3 meetings were organised for 36 participants including 16 men and 20 women in the Aru and Nyankunde health zones with a view to obtaining community commitment.

Community feedback focused particularly on the aggression against EDS teams and the monotony of the food in the Ebola Treatment Centers (ETCs). Several discussions were organized to clarify rumors and strengthen community support for the response.

Support for the pillars of the response : Surveillance: Facilitate contact tracing following a death in the Mongbwalu health zone. **IPC :** Prepare families to adhere to decontamination procedures and prepare relatives to cooperate.

North Kivu: Community dialogue with 38 community leaders on community-based surveillance with the support of community participation bodies for the prevention of Ebola virus disease (EVD) in the Vuhika health district; briefing of community health workers on the integration of community-based surveillance and the strengthening of infection prevention and control (IPC) measures in facilities for the prevention of EVD and nosocomial infections; briefing of 22 nurses and laboratory providers at Kyondo General Referral Hospital on risk communication and compliance with EVD prevention measures in hospital settings; capacity building of 16 members of health committees on community-based surveillance and strengthening of EVD prevention measures in the Butembo health zone; 34 motorcycle taxi drivers benefited from mass communication on the risk of EVD when transporting sick people to facilities in the Tuungane health district in the Beni health zone.

4.8. Logistics

Ituri

- Receiving inputs with WHO support; delivery of thermoflash to the Surveillance pillar; delivery of IPC inputs and medicines to the health zones of Nia-Nia and Mambasa; delivery of IPC inputs to the health zone of Rwampara and to the Red Cross.
- Continuation of development work at CTE HGR/Bunia, HGR/Rwampara and HGR Mongbwalu; monitoring of operational infrastructure at HGR/Bunia, CME/Bunia, HGR/Nyankunde, ISTM/Nyankunde, HGR/Rwampara, HGR/Aru and HGR Mongbwalu.

North Kivu

- Receipt of computer equipment from partner Unicef as part of support for the response against Ebola virus disease, consisting of 5 printers, 4 cartridges and 1 computer.

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- **Ituri :** PSEAH visibility materials were received for briefings and awareness-raising activities with the support of the UG-PDSS, including posters, leaflets, codes of good conduct and roll-up banners; awareness-raising activities were carried out with UNHCR investigators in Mambasa centre; an online meeting of the national PSEA coordination was held with actors involved in the response.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity of the CTEs and saturation in North Kivu (138.8%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3. Performance of the monitoring of contacts not optimal (87.1% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4 Low reporting of alerts in some affected areas	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not yet broken down by health zone (17 case)	Major gaps in the reconstruction of transmission chains	Conducting a data reconciliation, Remove any potential duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.). Active verification in the field.
6 Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (78 cases) HCW (including 18 deaths, case fatality rate 23.1%)	Urgent supply, strengthening of IPC training
8 Stockpile of unanalyzed samples (backlog) at the laboratory in North Kivu and delayed diagnosis	Delayed confirmation, high lethality (54.7%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities
9 Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10. Insufficient coordination and low utilization of COUSP frameworks	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, make operational the Provincial COUSP
11. Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12 Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13 Continuity of essential services (CEHS) compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14. High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

**FOR ALL
INFORMATION
ADDITIONAL,
PLEASE CONTACT**

For the National Institute of Public Health (INSP) of the DRC

The Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: diedonne.mwamba@insp.cd

Tel.: +243 816 040 145

The COU-SP Coordinator

Professor NGANDU Christian

Email: Christian.ngandu@insp.cd

Tel.: +243998091915

The Incident Manager

Professor Pierre AKILIMALI

Email: pierre.akilimali@insp.cd

Tel: +243815800288

With technical support from the World Health Organization (WHO) and
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